	Nursing action	Rationale
7	Maintain a tight fitting canopy whenever possible, provide nursing care through the sleeves or pockets of the tent.	Prevent oxygen leakage and disruption of the tent atmosphere.
8	Check child's temperature routinely.	Moisture accumulation may result in hypothermia.
9	No smoking sign should be pasted in the unit.	Oxygen helps in combustion.
10	Record the flow rate of oxygen, alteration in flow rate and child's reaction.	Serves as a communication between staff members.

Note:

1. Oxygen can be administered to babies using oxygen hood (Oxyhood).
2. Oxygen hood is a plastic device, which is kept over the head of the infant. It permits easy access to the child without loss of oxygen. It helps in efficient delivery of oxygen.
3. While placing hood over the head of the child, the edges of the hood should not rub against the child's chin, neck and shoulders.

Special Considerations

1. Mist is prescribed with oxygen therapy to liquefy secretions.
2. Humidified air may condense into water droplets on the inside walls of the tent, it is important to examine the child's clothing and bedding and change them as necessary to prevent chilling.
3. Electrical equipment used within or near the tent should be grounded properly.
4. It is preferable to monitor SpO2 of patent continuously.
5. Avoid the use of volatile, inflammable materials such as oils, grease, alcohol, either and acetone near the tent.
6. Nurses should be knowledgeable about the location and technique for using a fire extinguisher.
7. For the baby in oxygen tent, toys selected should be such that they retard absorption are washable and will not produce static electricity, e.g. woolen and stuffed toys. This ensures baby's safety.

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